

DEMO REFERRAL HOSPITAL BOARD

Comprehensive Bribery and Corruption Risk Assessment
Report

FY 2026/2027 - prepared using the EACC Practical Guide for Corruption Risk Assessment and Management (Appendix III(a) format)

Institution	Demo Referral Hospital Board (DRHB), Institution ID DEMO-005
Document reference	DRHB/ABC/CRA/2026-27/v1.0
Version / status	Version 1.0 - ACTIVE (no predecessor)
Assessment period	Fieldwork 17 - 28 August 2026; covers operations for FY 2026/2027
Approval	Recommended by the CPC at its Q1 meeting of 22 September 2026 (Min. CPC/04/Q1/2026-27); approved by the Accounting Officer on 24 September 2026
Effective from / to	24 September 2026 to 30 June 2027 (or until superseded)
Prepared by	IAO Committee, coordinated by Ms. Naomi Akinyi

Evidence category: **Risk-assessment evidence** (Sub-indicator 2). Submitted to the oversight portal on 29 September 2026, before the 30 September 2026 foundation deadline.

1.0 Context of the assessment

This assessment was conducted after the Hospital's Procedures for the Prevention of Bribery and Corruption were approved (Board Resolution DRHB/BD/RES/2026/044) and as a precondition for the FY 2026/2027 corruption prevention performance contract indicator. An FY 2025/2026 internal audit found that 71% of the value of quarterly pharmaceutical orders went to a single prequalified supplier, and the Customer Care desk received five complaints alleging payments for ward beds.

2.0 Scope: core and support functions

Category	Functional areas assessed
Core mandate	Patient admission and bed allocation; theatre and specialist clinic booking; pharmacy dispensing and medical stores; patient billing and fee waivers.
Support services	Procurement (quarterly orders under framework agreements); human resource (recruitment and locums); finance (supplier payments); ICT (hospital information system); medical equipment maintenance.

3.0 Methodology

The IAO Committee reviewed process maps, interviewed 42 staff across the nine functional areas, examined complaints and audit reports for FY 2025/2026 and held a validation workshop with heads of department on 28 August 2026. Each identified risk was placed in a High, Medium or Low risk level by consensus of the workshop, taking into account how often the risk was reported and the harm to patients and public funds.

Limitation of this assessment. Probability and impact were **not** scored separately on a declared numerical scale in this cycle, and no severity score (probability x impact) was computed. The risk levels in Section 4.0 are qualitative only. The IAO Committee will adopt the 1-5 probability and impact scales of the EACC Practical Guide in the FY 2027/2028 review (see Section 6.0).

4.0 Risk register (Appendix III(a): Corruption Risk Assessment)

Name of Institution: **Demo Referral Hospital Board** FY: **2026/2027** Corruption Risk Assessment

S. No	Functional area	Identified corruption risk (what could go wrong)	Risk source (why would the risk event occur)	Risk level (qualitative)	Existing controls
R-01	Procurement - quarterly orders for pharmaceuticals and medical consumables (Support)	Irregular procurement of medical supplies: quarterly framework orders concentrated with a favoured prequalified supplier in exchange for kickbacks or personal benefit.	Six suppliers prequalified, but the supplier for each order lot is chosen at the discretion of Supply Chain and the user department; no rotation rule. FY 2025/2026 internal audit found 71% of order value went to one supplier.	High	Tender Committee prequalification; e-procurement; conflict-of-interest declarations at prequalification.
R-02	Procurement - approval of requisitions, evaluations and purchase orders (Support)	Weak oversight of procurement approvals: purchases approved above delegated limits, without a required approval, or by an officer with a conflict, so irregular procurement passes unchecked.	Approvals recorded on paper files; nobody independently checks the approval chain. FY 2025/2026 internal audit found 6 of 40 sampled files lacked a required approval.	Medium	Delegation of authority schedule; IFMIS workflow covers payments only.
R-03	Pharmacy dispensing and medical stores (Core)	Diversion of drugs and medical supplies for private sale; collusive stock issues to wards.	Manual bin cards at sub-stores; issue vouchers reconciled only monthly; staff unaware of Procedures.	High	Monthly stock take; pharmacy issue vouchers; CCTV in main store.
R-04	Patient admission and bed allocation (Core)	Informal payments to secure a ward bed or private-wing room ahead of other patients.	Bed allocation by ward in-charges by telephone; no published bed state or waiting list.	High	Admissions register; Customer Care desk.
R-05	Theatre and specialist clinic booking (Core)	Bribes to move a patient up the elective surgery or specialist clinic waiting list.	Paper theatre lists kept by individual units; reasons for re-ordering not recorded.	Medium	Weekly theatre list approved by Head of Department.
R-06	Patient billing and fee waivers (Core)	Fee waivers or bill reductions granted in exchange for payment.	Waiver criteria applied with wide discretion; waiver office handles cash deposits.	Medium	Waiver Policy; dual signature on waivers above KES 50,000.
R-07	Human resource - recruitment and locum engagement (Support)	Nepotism or undue influence in shortlisting and in engaging locum clinicians.	Locum engagement by department heads without a panel; recusal not always minuted.	Medium	HR Policy; panel declaration forms for substantive posts.
R-08	Finance - supplier payments (Support)	Demands for facilitation payments to process supplier invoices; payment out of turn.	Payment queue not visible to suppliers; manual prioritisation by accounts staff.	Medium	IFMIS approval workflow; segregation of duties; internal audit.
R-09	ICT - hospital information system (Support)	Privileged users altering patient bills or deleting charges.	Shared supervisor log-ins in billing module; audit logs not reviewed.	Medium	Password policy; vendor support contract.
R-10	Medical equipment maintenance contracts (Support)	Inflated or fictitious maintenance call-outs.	Job cards signed by a single biomedical engineer.	Low	Annual maintenance contract; asset register.

5.0 Summary of results

Risk level	Risks	Count
High	R-01, R-03, R-04	3
Medium	R-02, R-05, R-06, R-07, R-08, R-09	6
Low	R-10	1

6.0 Assessment of existing controls and next review

Existing controls for R-01 depend on prequalification only and leave supplier selection for each order lot to individual discretion, so they cannot prevent concentration of orders. No one independently checks procurement approvals (R-02). Controls for R-03 and R-04 depend on paper records reviewed infrequently. Controls for finance and ICT are partly system-enforced (IFMIS) and are rated adequate subject to staff awareness of the new Procedures.

The FY 2027/2028 review will score probability and impact on the declared 1-5 scales of the EACC Practical Guide and compute severity scores. Until then, the qualitative risk levels above are used to prioritise mitigation.

7.0 Recommendations to the Mitigation Plan

- Introduce supplier rotation for quarterly orders under framework agreements, using a rotation list approved by the Accounting Officer, with a quarterly exception review of the rotation control from Q2 (treats R-01).
- Spot-check a sample of at least ten procurement approvals each quarter from Q2 and record the findings for the CPC (treats R-02).
- Train the named pharmacy and medical stores staff on the Procedures in Q1 (treats R-01 and R-03).
- Continue existing controls for R-05 to R-10 and monitor them at each quarterly CPC meeting; scope digital stock and bed-state controls for R-03 and R-04 in the FY 2027/2028 budget.
- Hold quarterly IAO Committee and CPC meetings to track implementation and approve reports to the Commission.

8.0 Approval

Name	Designation / role	Signature	Date
Ms. Naomi Akinyi	Chair, IAO Committee (prepared)		17 Sep 2026
Dr. Wanjiru Kimani	Accounting Officer (approved, on CPC recommendation Min. CPC/04/Q1/2026-27)		24 Sep 2026